

# AN INTRODUCTION TO IFS

---

Jack Engler

## BEGINNINGS

**I**ronically, I was introduced to one of the newest and fastest growing therapies in the United States in the First Parish Church of Cambridge, one of the oldest public buildings in the Boston area, on a cold, wet winter afternoon in 2005. I have come to view this as fitting because IFS has affinities with some of the world's great spiritual traditions. Although Boston has become the epicenter of growth for IFS, at the time it was an eastern outpost hosting a workshop given by the founder, Richard Schwartz.

I had been practicing psychodynamic psychotherapy in Cambridge for 25 years, so initially I wasn't all that enthusiastic about learning a new system. But this first exposure to IFS turned out to be compelling. I was impressed by Dr. Schwartz' presentation, by the basic concepts and structure of the IFS model, and by his description of how IFS emerged from a paradox he encountered while doing structural family therapy: "A family would make the structural changes the theory said should help them function better," I recall him saying, "but when I began to ask individual family members about their actual subjective experience of the changes the family had made, they would often say they didn't feel any better than before."

## PARTS AND THE MULTIPLICITY OF MIND

Exploring this paradox eventually led him to discover an *internal* family system similarly composed of separate components in complex relationships with each other, very much like members of the external family. That is, the mind is not a singular entity or self, but is multiple, composed of *parts*. This principle of multiplicity is at the core of the IFS model. Each of our parts, Dr. Schwartz found, has its own history, outlook and approach, its own idiosyncratic beliefs, characteristic moods and feelings, and its own relationships with other parts. More important, each part has its own distinct role or function within the internal system. A part, in other words, is not just a temporary emotional state or habitual thought pattern; it is a discrete and autonomous mental system with its own unique range of emotion, style of expression, set of abilities, desires and view of the world (Schwartz, 1995: 34). IFS views this multiplicity of mind as normal. In fact, we talk this way about ourselves all the time. We say, for instance, "A part of me wanted to do it. A part of me didn't." Assagioli's (1976) notion of "subpersonality," Jung's idea of "complexes" (1969) and Perls's (1969) gestalt therapy all capture

something of this idea of multiplicity. As psychoanalytic thinking has become more relational, it too has wrestled with the question of whether the self is singular or multiple (Mitchell, 1993; Engler, 2003). Although the idea that the mind is multiple is very old, the concept runs against our preferred way of representing ourselves to ourselves as a separate, autonomous self, an “I” or “me” whom we view as an independent center of consciousness and initiative. This idea of a separate, singular self is actually quite modern and western, only three or four hundred years old, a product of the European enlightenment (Taylor, 1989). So though Schwartz described arriving at his conceptualization independently, largely from asking clients a set of questions that were new to family therapists, IFS has distinguished antecedents.

Parts, as understood by IFS, cluster into three groups, each with a different function. The first is a protective group of parts called *managers*. These strategic, task-oriented parts strive to keep us organized and safe. They may push perfectionism, worry obsessively, exhaust with care taking, or more passively avoid, deny, discourage and devalue. Ironically, as they become extreme in the pursuit of safety and exile our injured parts to protect the system, they inflict more harm. The injured parts who get exiled are invariably burdened with emotional pain and dysfunctional beliefs about their worth and lovability that threaten the equilibrium protectors crave. When exiles override managers with emotional pain, they take us over, literally become us. IFS says they are now *blended* with us. Before we realize it, we identify with our exiles and take them to be who we are. We see ourselves and the world through their eyes and believe it is “the” world. In this state it won’t occur to us that we have been hijacked.

When exiles blend with us, another set of protective parts called *firefighters* are activated. Their role is to put out the emotional fire at any cost, often by starting backfires. This is the heavy artillery: alcohol, drugs, eating disorders, promiscuous sex, pornography, self-mutilation and suicidality, exacting a steep cost in collateral damage. Exiles, managers and firefighters are the trio we encounter in symptomatic behavior. Although exiles have been forced into a position not of their own choosing, protectors are volunteers who have often been playing their part since early in life, sometimes since infancy, to keep the system functioning.

From the point of view of IFS, therefore, many behaviors typically viewed as symptoms of underlying psychiatric disorders are actually strategies of protection. Common examples include anxiety-related symptoms such as obsessions, compulsions, phobias and panic attacks; or depression-related symptoms like passivity, withdrawal or insomnia and somatic complaints; or more self-destructive behaviors like eating disorders, self-harm and suicide. The IFS conceptualization of parts appealed to me because it is experience-near and nonpathologizing. Behaviors seem less entrenched and intimidating, more workable and open to change.

How are these parts actually experienced? Some experience them as internal people, though this may be the least common. More frequently they are experienced as internal “voices,” images or sensations. Contact with a part can be made through any mode of perception, including imagery, memory or physical sensation. They may manifest differently at different times, and be experienced as more or less distinct. But this doesn’t prevent connection with them, or make the work less effective. The key is approaching every part with genuine interest and respect.

Sooner or later most practitioners wonder what type of reality parts have. This question remains unresolved because IFS has developed, as with most psychotherapies, phenomenologically. Dr. Schwartz deliberately deferred the process of grounding IFS in theory and research until later to make IFS immediately available to clinicians. Research is now beginning, as are attempts to approach philosophical issues like the nature of parts.

## SELF

Dr. Schwartz introduced another finding in the introductory workshop that I have come to feel is the most important and innovative aspect of the entire IFS model: the notion of Self. This term *Self* (capital S) can be controversial because it carries certain connotations and invites associations with other therapeutic and spiritual systems that use the same term. Beginning with Freud, psychoanalysis has felt compelled to find a place for the concept of self. In fact it has become the most debated notion in contemporary analytic thinking (Mitchell, 1993). Long before IFS, I was familiar with a similar discussion in the great spiritual traditions, for which the term is central. “Self ” in the Upanishads or Vedanta, for instance, points to Ultimate Reality, Absolute Being, our innermost being behind all transient appearances and outside space and time. But there is no agreed upon meaning. The term is used inconsistently within the therapeutic and spiritual traditions themselves. Buddhism, for instance, uses the diametrically opposite term “No-Self ” for what is arguably the same reality (Engler & Fulton, 2012). But although certainly aware of its usage in the spiritual traditions (Schwartz, 2001, 29ff), Dr. Schwartz said he discovered what he came to call Self by listening closely to his clients.

He attempted to anchor this term, too, in direct experience. When a part steps back and releases its grip on us so that we no longer take it to be part of our core identity, what do we discover? First of all, a state of pure, open receptive awareness and acceptance without judgment or agendas. Self is what remains when parts are willing to unblend from us. When we relax into being that which is not any of our parts, IFS says we find our core, our essence, our true nature. Our natural state is a state of wholeness and completeness.

And from that core, uncontrived state, we discover that basic wholesome qualities emanate spontaneously. For instance, we don’t become compassionate and kind. Kindness and compassion are already there, as are many other positive attributes. We cannot acquire or generate them, and we don’t need to because they are innate. They are not transient states of consciousness that come and go, but timeless qualities of being. Self is like a beam of light that refracts into all the colors of the rainbow as it passes through a prism and illuminates our parts and external objects in the world. Interestingly, these “colors” overlap with many of the same qualities that all the Great Traditions identify. Dr. Schwartz used a mnemonic of Cs to identify eight of them, though this list is not exhaustive: calmness, clarity (or wisdom), curiosity, compassion, confidence, courage, creativity and connectedness. This is the same group of qualities that Buddhist teaching has long called *paramis*, or “perfections,” qualities of mind

considered essential to awakening. IFS uses the term “Self ” for this unblended state. The term Self-energy is also useful to designate the flow of feeling from Self to parts, and to recognize the different degrees to which a part is blended.

Two things about the notion of Self as described by Dr. Schwartz struck me as original. First, unlike most of the Great Traditions, which typically assume that years of disciplined practice are necessary to access Self-energy, practitioners of IFS find that any time a part becomes willing to unblend, we will experience some degree of inherent wisdom and compassion, some sense of freedom, lucidity and connection. At the same time, IFS recognizes the reality that our parts are seldom completely unblended from us. The part we are working with may need to stay partially blended to feel safe enough for interaction with us. Moreover, as one part unblends we may still be blended or partially blended with others. This means we are seldom completely “in Self.” Instead we move into Self by degrees. This idea may be original with IFS and is crucial in working with parts.

Second, the description Dr. Schwartz gave of Self playing an interactive role with parts was new and stimulating to me. In his conceptualization, Self doesn’t only witness or passively observe as in some meditation traditions; it has an active leadership role. One of his metaphors likens Self to the conductor of an orchestra, helping parts find new roles and function more harmoniously resulting in a symphony rather than a cacophony. From Self, curiosity, compassion and wisdom arise spontaneously, helping us to get to know and care for our parts. More surprising still was the idea that if we are even somewhat unblended, we are capable of providing the leadership our parts need.<sup>1</sup> The assumption—repeatedly borne out in my experience since—is that only Self-led leadership is trustworthy and effective. That which is whole and complete, aware and awake in us, is very unlike parts. It is unbiased, impartial and does not need things to be “this” way or “that” but instead expresses interest, concern, care and compassion. Parts have agendas. Self does not.

This was the overall view of IFS that Dr. Schwartz presented on that cold, wet winter weekend in Cambridge. I liked the experience-near conceptualizations and language. I also had been trying for some time to formulate a notion of self as both singular and multiple from the perspectives of psychoanalytic and Buddhist psychology (Engler, 2003). In addition, I was looking for fresh inspiration in my clinical work. I found it all that weekend.

As intrigued as I was with IFS theory at that workshop, what I remember most was a video of Dr. Schwartz working with a 21-year-old woman diagnosed with borderline personality disorder, a notoriously hard-to-treat set of symptoms. In the video she sat stiff and unmoving in an overstuffed chair, staring ahead, barely able to speak. An enraged suicidal part had vowed she would not live past 21 and this was her 21st birthday. The videotape showed Dr. Schwartz working with the client to get the suicidal part to unblend, help the exile it had been protecting to feel safe and heard and then, implausibly to me at the time, help the suicidal part actually find a different and less destructive role within her internal family. As I watched, I realized this work went to a level and used a skill set with which I was not familiar. I have since learned that after this session the young woman stopped self-mutilating and ceased to be suicidal.

# THE CAPE COD INSTITUTE: UNBURDENING

That summer of 2005, I attended a weeklong seminar that Dr. Schwartz offers annually at the Cape Cod Summer Institute. This seminar introduced me to *unburdening*, a process that may be unique to IFS. The notion that we accumulate burdens in the form of extreme ideas or feelings seemed self-evident to me. However, the notion that burdens are not intrinsic to parts and can be released was not. For unburdening to occur, both client and therapist must first recognize that the burden is not the part's essence, that it was imposed from the outside at some point in the individual's life, and that it need not evoke guilt or shame. Next, the part needs to have some or all of its experiences witnessed and understood by the client's Self. Only then will it let the burden go. IFS offers a stepwise protocol for the release of burdens, in which parts are invited to choose their own rituals for letting go; on the other hand, unburdening often occurs spontaneously. Dr. Schwartz acknowledged that some clients find the concept too gimmicky and easy. Or else they have trouble believing that burdens can actually be released. For others, the idea flies in the face of their all-too-long struggle with burdened parts. Still others carry a secret (or unconscious) belief that there cannot really be an end to internal conflict and suffering. Even some IFS therapists have trouble believing unburdening is possible. Among those who accept the notion, there is controversy over whether an unburdening will hold over time or will have to be repeated. In my experience, if a part carries several different burdens, or has received the same burden from several sources, the process often does need to be repeated. My first unburdening, described below, was one and done, but I know from subsequent experience this is not always the case.

*I returned to my room in late afternoon after a day at the seminar introducing the unburdening process. I had a couple of hours before supper and thought I'd use the time to practice what we'd been taught. I mostly wanted to see for myself whether and how it worked. Soon after starting, it became very real. When I went inside to see which parts were asking for attention, I quickly became aware of a continuous high-pitched scream of terror, which seemed to come from the back of my mind. I gradually realized that I had been hearing this screaming for as far back as I could remember. It was in the background of almost every state of consciousness. I invited the part who was screaming to come forward. It was a 7-year-old boy. I asked if there was something he wanted me to understand about his screaming.*

*At once a memory surfaced of a terrifying hospital experience in which this child was strapped to a gurney while strangers slowly lowered an ether mask over his face. His parents had disappeared. All he could see was the black, foul-smelling hole coming closer and closer to engulf and suffocate him. Whether his scream was out loud or only in his mind, he didn't know, but he did suddenly know why he was screaming. I asked him if there were more he needed me to witness, or if he could finally release this burden of terror, shame, abandonment and helplessness. He said he felt ready. I asked if he wanted to come into the present to be with me and find a role in my current life. Again he said yes. I told him he could go ahead and release the burden and join me, which he did. I have not heard the screaming since.*

# INDIVIDUAL THERAPY

I didn't have to wait long before discovering further applications of IFS. Later that fall of 2005, I was hit broadside while driving. Fortunately the collision was low impact. Neither of us seemed hurt. Two to three weeks later, however, I began to experience pain in my neck. Over the fall and winter the pain gradually radiated down my left side until it became unbearable. A series of imaging studies revealed problems in my cervical spine. After a series of epidurals in the spring of 2006 remitted most of the pain, I approached one of Dr. Schwartz' senior assistants from the Cape Cod workshop about doing individual IFS therapy. This decision proved fortuitous, especially after a consulting neurologist told me—without sufficient data, I thought—that he “would have to think about ALS,” or Lou Gehrig's disease. Hearing this threw many of my parts into panic, especially exiles who were constantly on the verge of feeling overwhelmed by fear. Initially, trying to help them un-blend was next to impossible. So I went back into therapy again, this time with a clear understanding of the task and how to approach it.

I've sought help from therapists a number of times over the past thirty years, but I experienced this two-year therapy as completely different. I have no doubt it was this therapist and the IFS approach we took that saw me through. It was the work of slowly unblending from terrified parts, befriending and reaching out to frightened protectors, and being able to work with them all from Self, that enabled me to gradually find some balance and inner peace. Even in daily mindfulness meditation, which I had been practicing for 35 years, I found myself spontaneously working with these terrified parts. Although I would not claim that IFS was the only therapeutic approach that would have worked, it proved remarkably effective in this extreme situation when living and dying seemed to hang in the balance. I can feel no deeper gratitude.

# IFS IN DAILY LIFE

I include the following vignette because it shows how parts work can be done in increments, on the spot, in the middle of daily life; and because it was one of many similar experiences that engendered my trust in the method. It also shows some of the steps involved in working with a part. I had been invited to give a series of talks at four different hospitals and colleges in the spring of 2007. I had a bad feeling going into them for reasons I didn't fully understand. And indeed the first three did not go that well, each one a little less successful than the one before. I could feel my anxiety rising and my confidence sinking with each talk. I became particularly anxious as the fourth one approached, feeling a kind of dread. Why I didn't think to consult my parts earlier I don't know—probably because they were so blended with me. I finally remembered to contact the part who was making me so anxious. The part responded immediately. What it said made a certain sense.

“If you go down to New York feeling this anxious, you're going to make a fool of yourself in front of your professional colleagues. You know what will happen—you'll feel humiliated and ashamed. My job is to protect you from embarrassing yourself. I figure if I can make you anxious enough, you'll find some way to excuse yourself and not go. I will have spared you the shame.”

I thought about this and replied, “Thank you for wanting to help. I can see you intend well. However, I committed to giving this talk. If you persist in making me more anxious and I mess up the talk, you’re going to bring about the very situation you’re trying to protect me from. Do you see that?”

The part immediately replied, “Oh, I didn’t think about that! That’s not what I intended! What can we do?”

I said, “Let me make a suggestion. Why don’t you accompany me to New York? There’s no point in asking you to trust me. Come see for yourself whether you need to be as protective as you think. Your over-protectiveness is partly what’s creating the problem.”

The part agreed. I made an additional request: “You might become anxious yourself on the way down and jump in to try to prevent me from continuing. But if that happens, we’ll know what that’s about and we’ll deal with it so we can continue.” The part agreed to this too.

The flight to JFK was uneventful. I was feeling no more than the normal anxiety I would expect. However, when I went to get a ticket for the airport shuttle to the Long Island Railroad, I suddenly became disoriented and confused. Complete brain fog. Everyone else was getting their ticket and streaming through the turnstiles, and I couldn’t figure out how to operate the automated ticket machine I’d never had a problem with before. A moment of clarity: “Is that you? Did you just jump in to prevent me from getting there?”

“Yes, it’s me.”

“OK. You’re still having problems trusting me so you’re taking matters into your own hands again. No problem. Please step back so we can proceed. But keep watching and observing.” Instantly the fog cleared. I knew exactly what to do. An hour later I came out of the subway, felt the cold mix of sleet and rain on City Hall Plaza, saw the inches of slush underfoot, and in a moment realized I’d left my umbrella on the train.

“Is that you again?”

“Yes, my anxiety got the better of me again. This was my last chance to stop you from trying to give the talk.”

“Thank you. We anticipated this might happen. Please step back and let me cross the Plaza. But come with me and witness the talk. See if I make a fool of myself. I’ll check in with you afterward to see what you think.”

I turned around and discovered a kiosk selling umbrellas for \$5 directly behind me. I bought one, crossed City Hall Plaza, and gave the talk—not my best, but I enjoyed giving it and enjoyed the students. I did not embarrass myself. On the way back to Grand Central Station, I checked in with the part as promised.

“What did you think?”

In reply, a deep, quiet, contented silence, which lasted all the way back to Boston. Since then I’ve certainly had performance anxiety before talks, but only once before this had I experienced the dread I felt that spring, and never the inner peace I felt afterward.

# CLINICAL PRACTICE

I began using the frame and language of IFS in 2007. Often I use the entire therapy protocol if the client is open to working directly with parts. With some explanation and encouragement, this seems to happen easily and naturally, sometimes for the entire session. In daily living I use IFS all the time. For me, this is in some ways the most interesting and fruitful application of all. Parts don't wait—sometimes can't wait—until the therapy hour to make their presence felt and have their needs acknowledged. I've also found IFS very compatible with mindfulness and have made it part of my daily meditation practice. Having a meditation practice and using IFS helps clients to work interactively with the parts they discover when they “go inside.”

## INTRODUCTION TO THE CHAPTERS

In the following chapters the reader will notice the repeated appearance of the guiding axioms of IFS. Parts always intend our well-being. This is true regardless of how it may look when we first encounter a part and no matter how misdirected its actions. It is true even of aversive and afflictive mind states like the “Three Poisons” of greed, hatred and delusion in Buddhist psychology. Because all parts have some Self-energy at their core, they themselves seek balance and harmony and are ultimately open to change. Unskillful or harmful behavior derives from the extreme role a part has been forced into, not from its intentions. The goal of therapy is therefore not to eliminate parts but to help them transform. And when they do transform and change roles, they remain part of the internal family. They do not disappear.

The essential element for the transformation of parts is Self-energy; the key to accessing Self-energy lies in helping parts to unblend; and the key to unblending is curiosity and disinterested inquiry into their experience, with no intention to fix, change or eliminate. As the ground state of receptive awareness, Self provides the compassionate leadership that parts need to initiate change. It is a given in this treatment that burdens, including the burdens of feeling shamed and unlovable, can be released with the help of Self-energy. In sum, the process of IFS involves several steps, in this order: unblending, befriending, being curious about and transforming parts. The befriending of parts is fundamental. *All parts.* This is a tantric practice: adversaries become allies spontaneously. As the IFS mantra states, *All parts welcome!*

These chapters are intended both for those who are familiar with the model and for newcomers. Although serving as a rich introduction, the book also provides insight into the current thinking and practice of some of the most experienced IFS therapists. Appropriately for a model that is experiential by design, each author invites you into clinical sessions, offering a direct glimpse into the internal experience of the client, and often the therapist as well. Some chapters describe using IFS with specific client populations: trauma and dissociative disorders, pornography addiction, children and adolescents, chronic pain and disease. Others describe the use of IFS in different therapeutic modalities: couple



therapy, psychopharmacology and health coaching. And others illustrate using IFS in relation to sexuality and shame.

The work of these authors makes it clear that IFS is continuing to evolve as a system of healing and a theory of mind. As Dr. Schwartz spells out in his pivotal chapter on the therapist–client relationship, IFS is not only finding new applications, but is discovering new dimensions of the model itself. He acknowledges he emphasized the relationship between the client’s Self and parts as the primary mode of healing at the outset of developing IFS, giving much less attention to the relationship between therapist and client. As a result, though relational in multiple dimensions, IFS has sometimes been perceived as a nonrelational therapy. He describes himself as now having come full circle: from minimizing the role of the therapeutic relationship to appreciating the ways in which it is foundational.

In conclusion, each contributor to this volume hopes to stimulate your curiosity, broaden your options and perhaps inform your own internal family.

## NOTE

1. [This is reminiscent of what Buddhist psychology calls \*saṃsaṅkarikachitta\*](#), unmotivated or spontaneous action that is appropriate and commensurate with the needs of the situation that prompted it, and that is not attached to a particular outcome.

## REFERENCES

- [Assagioli, Roberto](#). 1976. *Psychosynthesis*. New York, NY: Penguin.
- [Engler, Jack](#). 2003. “On Being Somebody and Being Nobody.” In *Buddhism and Psychoanalysis: An Unfolding Dialogue*, edited by Jeremy Safran, pp. 35–100. Somerville, MA: Wisdom Publications.
- [Engler, Jack, & Paul Fulton](#). 2012. “Self and No-Self in Psychotherapy.” In *Wisdom and Compassion in Psychotherapy: Deepening Mindfulness in Clinical Practice*, edited by Christopher Germer & Ronald Siegel, pp. 176–188. New York, NY: Guilford Press.
- [Jung, Carl](#). 1969. “The Structure and Dynamics of the Psyche.” *Collected Works*. Vol. 8. Princeton, NJ: Princeton University Press.
- [Mitchell, Stephen](#). 1993. *Hope and Dread in Psychoanalysis*. New York, NY: Basic Books.
- [Perls, Fritz](#). 1969. *The Gestalt Approach and Eyewitness to Therapy*. New York, NY: Bantam Books.
- [Schwartz, Richard](#). 1995. *Internal Family Systems Therapy*. New York, NY: Guilford Press.
- [Schwartz, Richard](#). 2001. *Introduction to the Internal Family Systems Model*. Oak Park, IL: Trailheads Publications.

[Taylor, Charles](#). 1989. *Sources of the Self: The Making of the Modern Identity*. Cambridge, MA: Harvard University Press.